

Colonoscopy Clinical Care Standard

The Colonoscopy Clinical Care Standard aims to ensure the safe and appropriate use of colonoscopy, and to maximise patients' likelihood of benefit from the procedure while reducing their risk of avoidable harm.

What is a colonoscopy?

Colonoscopy refers to the examination of the entire large bowel using a colonoscope – a camera on a flexible tube. It is a complex task that requires the colonoscopist to manipulate the colonoscope effectively to visualise the bowel, while performing therapeutic interventions such as removing polyps or taking tissue samples when required.

Colonoscopy is often performed as a diagnostic intervention to investigate possible bowel cancer, either in people with symptoms and signs of bowel disease or those with an increased risk of bowel cancer. It may also be used to help diagnose the cause of symptoms in conditions such as inflammatory bowel disease.

About the Standard

The Standard includes:

- eight quality statements describing safe and appropriate care
- a set of indicators to support monitoring and quality improvement

We also have resources for clinicians, healthcare services and consumers to support the implementation of the Colonoscopy Clinical Care Standard



Colonoscopy Clinical Care Standard

PDF

3.1 MB

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Quality Statements

Quality Statement 1 – Initial assessment and referral

When a patient is referred for consideration of colonoscopy, the referring clinician provides sufficient information for the receiving clinician to assess the appropriateness, risk and urgency of consultation. The receiving clinician or service allocates the patient an appointment according to their clinical needs.

For clinicians

Clinicians making referrals

When referring patients for consideration of colonoscopy, provide a comprehensive referral to enable accurate assessment of the patient's suitability for, and urgency of, colonoscopy. Standard (electronic) templates can help. See the Commission's [Colonoscopy Referral Information](#) for further information, and refer to local HealthPathways where relevant. The referral should include:

- the indication for the referral, including presenting symptoms and the clinical concern
- results and dates of previous investigations, including iFOBT (indicating whether this was through the National Screening Program), colonoscopies and histopathology
- all relevant medical and family history, including of bowel and other cancers and known genetic predispositions
- current medicines and other medical conditions
- previous relevant treatment.

Consider the indications and surveillance intervals recommended in current evidence-based guidelines such as the Cancer Council Australia's *Clinical practice guidelines for the prevention, early detection and management of colorectal cancer*,

and *Clinical practice guidelines for surveillance colonoscopy*. Evaluate the likelihood of the patient benefiting from the procedure, considering their overall health, comorbidities, likelihood of benefit from future management, the procedural risk and their willingness to proceed. Discuss with the patient whether the specialist receiving the referral will assess them individually before undertaking the colonoscopy. Give the patient information about what to expect, including whether there are likely to be costs. Provide an opportunity to ask questions and have them answered.

If referral to open-access services is an option, ensure the patient is suitable according to the local service intake guidelines and considering the patient's age, general health, comorbidities and ability to take bowel preparation independently. Provide clear instructions to the patient on what they need to do to act on the referral, the degree of urgency, and what to do if they cannot get an appointment in the recommended timeframe.

Clinicians receiving referrals

When receiving referrals for colonoscopy, ensure that your protocols and processes allow for reviewing and determining appropriateness for the referral, and for allocating appointments based on clinical need. Clearly communicate the required referral information to referring clinicians, preferably using a standardised template.

For healthcare services

Healthcare services supporting referrals

Use consistent processes to ensure that referrals are accurate and comprehensive to enable assessment and prioritisation. Use an appropriate template, preferably electronic. See the Commission's [Colonoscopy Referral Information](#) for further information.

Healthcare services receiving, allocating or prioritising referrals

Ensure that clear referral guidelines are available for referring clinicians, identifying the type and format of clinical information required. Ensure that processes support the provision of services according to the patient's clinical priority. Using agreed, standardised templates, preferably electronic, can assist the communication of important information between referring clinicians and colonoscopy services.

For open-access services, processes and procedures should ensure adequate consideration of the patient's comorbidities, current medications, risks and suitability for the procedure and:

- include a process to assess the suitability of the patient for a direct-access procedure

- include a process to obtain the patient's relevant information, including the dates and findings of previous colonoscopies
 - provide the opportunity for contact between the patient and a suitably trained clinician (including a registered nurse or nurse practitioner) before the day of the procedure to enable the patient to ask questions about the procedure, including about potential risks, benefits, and bowel preparation; this may be via telehealth when appropriate.
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For consumers

People might have a colonoscopy for different reasons and every person's situation is different. Just because you are referred to a specialist to consider having a colonoscopy does not mean that it will be the right thing for you.

It is important that the clinician or healthcare service that you are referred to has the right information about you and your medical history. This will help them decide if a colonoscopy is likely to help you. It may also help them decide how soon to book your appointment. The referral document should include:

- your current and past medical conditions
- your age
- your family medical and cancer history
- current medicines
- the results of previous tests, imaging and colonoscopies.

The clinician who gives you the referral will explain what to expect once the service has received your referral. In most cases, you will have a consultation with the clinician you are referred to before any procedure is booked. However, you may be referred to an open-access colonoscopy service if this is a suitable option for you. Open-access (sometimes called direct-access) means you will be booked for the procedure without having a consultation with the clinician performing your procedure beforehand.

What is open-access colonoscopy?

An open-access (sometimes called direct-access) colonoscopy service is a service which allows clinicians to refer patients for a colonoscopy without a prior consultation with the colonoscopist. Open-access models have been developed to improve access to services for suitable patients (such as following a positive screening iFOBT ['poo test']).

Cultural safety and equity

Explain the rationale for assessment, tests and interventions to the patient and their family, carer, or support people in a culturally safe way.

Recognise and address potential barriers to people accessing care, such as language differences or being from a remote or disadvantaged community (see Communication and person-centred care for further information).

Consider actions to help reduce wait times and streamline referrals, such as:

- ensure processes to capture and to act on identification data
- ensure referrals and service intake processes provide an opportunity to self-identify
- provide culturally appropriate and codesigned information resources (in local language as appropriate) and the opportunity to have questions answered with a trusted health professional
- develop streamlined referral pathways, particularly for those from rural or remote communities, and liaise with primary care clinics including ACCHOs to ensure travel arrangements are in place.

Related resources

For clinicians and healthcare services

- [Clinical practice guidelines for the prevention, early detection and management of colorectal cancer](#) - Cancer Council Australia
- [Clinical practice guidelines for surveillance colonoscopy](#) - Cancer Council Australia
- [Culturally appropriate bowel screening resources](#) - SA Health

Quality Statement 2 – Appropriate and timely colonoscopy

A patient is offered timely colonoscopy when appropriate for the investigation of signs or symptoms of bowel disease, surveillance or screening, as consistent with national evidence-based guidelines. Decisions are made in the context of the patient's ability to tolerate the bowel preparation and colonoscopy, and their likelihood of benefit. If colonoscopy is not appropriate, the receiving clinician advises the patient and their referring clinician of alternative recommended management.

For clinicians

Consider whether colonoscopy is indicated for the patient according to national evidence-based guidelines, the epidemiology of disease (including bowel cancer and inflammatory bowel disease) and how findings on colonoscopy are likely to influence management. Assess the likely benefits to the patient, as well as the risks associated with the bowel preparation, sedation, the procedure itself, any further management and the risks associated with not having the procedure.

Refer to Cancer Council Australia's *Clinical practice guidelines for the prevention, early detection and management of colorectal cancer*:

- for people with symptoms suggestive of bowel cancer or a positive iFOBT
- for people who are at markedly higher than average risk for bowel cancer such as those with familial syndromes, where screening colonoscopy is recommended.

For people requiring surveillance colonoscopy (including for inflammatory bowel disease), refer to Cancer Council Australia's *Clinical practice guidelines for surveillance colonoscopy* regarding the frequency and surveillance intervals for colonoscopy in high-risk individuals.

Ensure that colonoscopy is triaged and scheduled according to relevant and locally approved triage criteria, that reflect Cancer Council Australia's guidelines. If colonoscopy is not appropriate, advise the patient and their referring clinician about recommended alternative diagnostic strategies or management.

For healthcare services

Ensure that policies and processes support the timely and appropriate provision of colonoscopy. This includes:

- supporting and promoting clinicians' use of national evidence-based guidelines, including Cancer Council Australia's *Clinical practice guidelines for the prevention, early detection and management of colorectal cancer* and *Clinical practice guidelines for surveillance colonoscopy*
- supporting and encouraging clinician participation in quality improvement and peer-review processes.

For healthcare services that receive referrals, ensure that policies and procedures for triage and scheduling of colonoscopy appointments for bowel cancer-related indications reflect guideline recommendations from the Cancer Council Australia in regard to timeliness of follow-up or investigation. Consider state and territory-based colonoscopy categorisation criteria, such as NSW Agency for Clinical Innovation's [NSW colonoscopy categorisation](#) and Victoria Department of Health's [Colonoscopy categorisation guidelines](#), as relevant to the setting.

For consumers

Colonoscopy is used when clinicians want to look at the inside of the bowel to check for signs of disease. It may be recommended:

- if you are experiencing certain bowel problems
- to follow up a previous bowel condition
- because of test results (such as a CT scan or iFOBT [‘poo test’])
- because of your family history or having a gene mutation such as Lynch Syndrome.

You should only be offered a colonoscopy if the benefits outweigh any risks of the procedure for you. While most people do not have any complications, the bowel preparation, sedation, and the colonoscopy all have some risks. Additionally, the process may involve time commitments, travel, and associated costs. Your clinician will discuss these risks with you, considering your general health. You should also talk about the risks of not having the colonoscopy. For some people a colonoscopy may need to be carried out as soon as possible, while for other people it may need to be done less urgently. If a colonoscopy is not recommended, then the clinician may suggest an alternative test.

Related resources

- [Optimal care pathways: Colorectal cancer](#) – a framework for delivery of high-quality evidence-based care for people with cancer – Cancer Council Victoria and Department of Health Victoria
- [NSW colonoscopy categorisation](#) – NSW Agency for Clinical Innovation
- [Colonoscopy categorisation guidelines](#) – Victoria Department of Health
- [Clinical practice guidelines for the prevention, early detection and management of colorectal cancer](#) – Cancer Council Australia
- [Clinical practice guidelines for surveillance colonoscopy](#) – Cancer Council Australia

Indicator

Indicator 2a	Evidence of a locally approved policy that ensures the timely and appropriate provision of colonoscopy
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Quality Statement 3 – Informed decision making and consent

Before starting bowel preparation, a patient receives comprehensive patient-appropriate information about bowel preparation, the colonoscopy, and sedation or anaesthesia. The patient has an opportunity to discuss the reason for the colonoscopy, the risks, benefits, financial costs and alternative options before deciding to proceed. Their understanding is assessed, and the information provided and their consent to sedation, colonoscopy and therapeutic intervention is documented.

For clinicians

Provide the patient (or their responsible decision maker where relevant) with clear and comprehensive information about all aspects of the colonoscopy relevant to the patient's decision and consent. Include information about the bowel preparation, the use of sedation (or anaesthesia), the colonoscopy and any therapeutic interventions. Use language that they can understand. Arrange an interpreter if required.

Inform the patient of:

- the reason for the colonoscopy
- its likelihood of benefits
- potential adverse events, including those related to the bowel preparation or sedation, perforation, bleeding (immediate and delayed), splenic injury and missed pathology
- what happens during a colonoscopy
- the financial costs
- the alternatives to having the colonoscopy, including any risks of not having the colonoscopy.

Information on potential adverse events, risks and benefits should be individualised and relevant to the patient. Provide adequate time for the patient to consider the information provided and to ask questions before consenting. Respect the patient's decision and document it and their informed consent in the medical record, with a description of the information discussed and provided to the patient.

For healthcare services

Ensure that clear, written information is available to patients for all aspects of the colonoscopy for which the health service organisation is responsible. This may include information about bowel preparation, the colonoscopy and associated sedation or anaesthesia.

When consent is being obtained, ensure protocols and procedures enable patients to receive adequate information to inform their decision, are supported to ask questions, and provide consent before the start of bowel preparation. Ensure interpreter services are accessible, and their use is supported. Ensure policies and procedures support the principles and practices of informed consent and appropriate documentation.

For consumers

If your clinician recommends that you have a colonoscopy, you will need to decide whether to go ahead with it. To help you make your decision, your clinician will explain all parts of the process to you, including:

- bowel preparation – the process for clearing your bowel before the colonoscopy using medicines, changing your diet and fasting (not eating for a period of time)
- sedation – medicines given to minimise discomfort during the colonoscopy
- the colonoscopy procedure – how the colonoscope is used to look at your bowel, and to help remove polyps or tissue samples
- what to expect after the procedure.

The discussion will include:

- why the clinician is recommending a colonoscopy
- benefits to your health
- risks of the bowel preparation, sedation and the colonoscopy
- risks of not having the colonoscopy
- what happens during a colonoscopy
- any out-of-pocket costs
- any alternatives to colonoscopy.

The decision about whether to have a colonoscopy is yours. You can ask for time to make your decision. If you decide to have the colonoscopy, you will be asked to give consent. Giving consent means that you understand what is involved in having the colonoscopy, what the risks and benefits are for you, and that you agree to have the colonoscopy. It is important that you ask questions if you need more information before you make your decision. This should happen before you start the bowel preparation. If you need an interpreter or any other assistance with communication, this can be arranged. If you choose to have the colonoscopy, your consent will be recorded in writing. Even after you have given your consent, you can ask for more information or change your mind about having the colonoscopy at any time before the colonoscopy begins.

Cultural safety and equity

Provide written and visual information in a way which reflects the literacy, language, and cultural needs of the individual patient and builds understanding, engagement and empowerment. Written material for Aboriginal and Torres Strait Islander populations should be developed in partnership with the community and people with expertise in Aboriginal and Torres Strait Islander health issues.

Include family, kin, community members or other trusted healthcare providers in discussions, if the patient desires this. Allow time to build rapport and trust. Consider the need for multiple appointments and methods of communication. Explore and address any concerns or stigma associated with the potential diagnosis, such as for cancer.

Related resources

For consumers

- [Information about colonoscopy](#) – a fact sheet for patients – Gastroenterological Society of Australia [GESA]

Quality Statement 4 – Bowel preparation

A patient booked for colonoscopy receives a bowel preparation product and dosing regimen individualised to their needs, comorbidities, regular medicines and previous response to bowel preparation. The importance of good bowel preparation for a quality colonoscopy is discussed with the patient. They are provided with consumer-appropriate instructions on how to use the bowel preparation product and their understanding is confirmed.

For clinicians

Provide written and verbal consumer-appropriate information to patients preparing for colonoscopy, using interpreter services where necessary. Select an appropriate bowel preparation agent and ensure the patient knows how to obtain and use it, considering individual risks, comorbidities, current medicines, and the patient's previous experience with bowel-cleansing medicines. Clearly explain the purpose of bowel preparation, the importance of following the prescribed procedure, the regimen and the potential side effects of the bowel preparation products. Allow the

patient appropriate time to ask questions and confirm that they understand what to do and its importance. Let the patient know who they can contact if they are unclear about what to do.

A split-dose regimen results in a higher quality colonoscopy examination compared with ingestion of the entire preparation on the day or evening before the colonoscopy and has been associated with increased adenoma detection rates. Typically, this involves splitting the standard dose of the bowel preparation between the day before and the morning of the colonoscopy. Ensure that some of the bowel preparation dose is given on the same day of the procedure (3–6 hours before the planned start of the procedure).

Ensure patients on diabetes medicines, anticoagulants, antiplatelets, or other medicines are provided with individualised instructions about how to adjust their medicines and manage their condition as they undergo bowel preparation. Enquire about the patient's use of medicines to lose weight, due to the potential complications in endoscopic procedures (see [Related resources](#) for further information). Consider whether a patient with relevant comorbidities needs specific health or personal support while undergoing bowel preparation. For example, overnight admission for patients who are unlikely to manage bowel preparation independently.

For healthcare services

Ensure that policies and procedures support best practice for bowel preparation. Support patients by enabling access to information about bowel preparation. Healthcare services with responsibility for providing bowel preparation and advice should ensure processes to:

- provide clear, written patient information about the bowel preparation procedure
- provide access to interpreter services or translated materials
- provide a telephone number for enquiries patients may have during bowel preparation
- enable clinical staff to periodically review and approve patient information.

Where relevant to the facility, ensure policies support providing extra assistance to patients who are unlikely to manage bowel preparation independently, including overnight admission if needed.

For consumers

Before you have a colonoscopy, you need to make sure your bowel is as clear as possible. If your bowel is not clear, polyps or even cancers may be missed, or you may need to have the colonoscopy again. This means it is important for you to follow the

instructions carefully and ask questions if you do not understand what to do. To get your bowel ready for the colonoscopy, you will be:

- given instructions about what (and what not) to eat and drink
- advised when to drink extra fluids to stop you from getting dehydrated
- given, or asked to buy, medicine to clear out your bowel by causing diarrhoea.

Make sure you understand when to take the medicines, usually starting the day before the colonoscopy. Your clinician will explain how these medicines may affect you. You should tell them about any previous experience you have had with bowel preparation.

Preparation for colonoscopy can also affect your other health conditions or medicines, such as medicines for diabetes, weight loss or to prevent blood clots. You may need to stop or change the way you take your other medicines or follow special instructions in the days before your colonoscopy. Check with your clinician about all your usual medicines. They will discuss any changes you may need to make. During bowel preparation, some people may need extra personal or health support and a few may need an overnight stay in hospital.

If at any time during the bowel preparation you are unsure what to do, ring your clinician or clinic to check.

Cultural safety and equity

Provide culturally appropriate written and visual instructions on what to do for bowel preparation before a colonoscopy.

Allow time to yarn about why it is needed and what to expect, and consider any complicating factors (such as living arrangements or the need to travel).

Related resources

For consumers

- [Information about bowel preparation](#) – a fact sheet that provides information for patients having a colonoscopy – GESA

For clinicians and healthcare services

- [Sodium glucose co-transporter 2 inhibitors | Safety advisory – diabetic ketoacidosis and surgical procedures](#) – Therapeutic Goods Administration [TGA]
- [Periprocedural diabetic ketoacidosis \(DKA\) with SGLT2 inhibitor use in people with diabetes](#) – Australian Diabetes Society [ADS]
- [Clinical Practice Recommendations regarding patients taking GLP-1 receptor agonists and dual GLP-1/GIP receptor co-agonists prior to anaesthesia or](#)

sedation for surgical and endoscopic procedures - ANZCA, GESA, National Association of Clinical Obesity Services [NACOS], ADS

Quality Statement 5 – Sedation

Before colonoscopy, a patient is assessed by an appropriately trained clinician to identify any increased risk, including cardiovascular, respiratory or airway compromise. The use of sedation is planned accordingly. The risks and benefits of sedation are discussed with the patient. Sedation is administered and the patient is monitored throughout the colonoscopy and recovery period in accordance with Australian and New Zealand College of Anaesthetists guidelines.

For clinicians

Provide patients with the opportunity to discuss the approach to sedation. This may include the option of no sedation where this is the patient's preference and is clinically appropriate.

Ensure that the patient's suitability for sedation is assessed in advance of the colonoscopy by a clinician who is appropriately trained to make such an assessment. This should include assessment of any increased risks such as cardiovascular, respiratory or airway compromise.

Ensure that the facility is appropriate for the patient, taking into account their clinical requirements and comorbidities, as described in ANZCA's *Guideline for the perioperative care of patients selected for day stay procedures (PG15)*. If an increased risk is identified, an anaesthetist, or other trained and credentialed medical practitioner within their scope of practice, should assess the patient and be present during the colonoscopy to care for the patient.

Consider whether the patient is likely to require overnight admission or increased medical support and whether admission to a day procedure unit is appropriate. The sedationist should discuss the risks and benefits with the patient and obtain their informed decision and consent. Ensure that the patient understands that their awareness of the colonoscopy will depend upon the depth of sedation.

Sedation must be administered by a credentialed practitioner working within their scope of practice. Both anaesthetist and non-anaesthetist clinicians should provide sedation as described in current ANZCA guidelines with respect to:

- the number of staff present during the sedation and their level of training, competence and scope of clinical practice
- facilities, equipment and medicines
- administration of sedation
- monitoring of patients during the colonoscopy and in the recovery room.

Note: Whilst anaesthetists are specialists in the sedation–anaesthesia continuum, sedation may be administered and managed by clinicians from other clinical disciplines. ANZCA’s *Guideline on procedural sedation (PG09)* outlines the requirements for non-anaesthetist sedationists managing minimal or moderate procedural sedation. (See sedationist in the Glossary for further information). For anaesthetists targeting deep sedation or general anaesthesia, the patient should be managed by a medical practitioner trained and credentialed to provide anaesthesia. Refer to ANZCA’s *Guideline for the perioperative care of patients selected for day stay procedures (PG15)* and *Position statement on roles in anaesthesia and perioperative care (PS59[A])*.

For healthcare services

Sedation and anaesthesia should be provided in accordance with current ANZCA recommendations such as the *Guideline on procedural sedation (PG09)*, *Guideline for the perioperative care of patients selected for day stay procedures (PG15)* and *Position statement on informed consent for anaesthesia or sedation (PS26)*.

Ensure that local policies and procedures are in place, and services adequately resourced, to implement the ANZCA guidelines. Policies should ensure that pre-sedation assessment is carried out by appropriately trained clinicians to identify patients who are not suitable for intravenous sedation in the absence of an anaesthetist, and to plan for sedation accordingly. Policies should include arrangements for providing colonoscopy without sedation where the managing colonoscopist has assessed this as clinically appropriate for the patient and it is the patient’s preference.

Ensure that clinicians who administer sedation or anaesthesia for colonoscopy are credentialed by the health service organisation and are operating within their defined scope of clinical practice. Ensure that clinicians maintain their skills by participating in ongoing professional development and review of performance. Ensure sedationists achieve the Safe Sedation Competencies as described by ANZCA. Implement and ensure compliance with policies and procedures for the safe supervision of trainees, where relevant to the facility.

For consumers

Just before starting your colonoscopy, you will be given medicines to minimise your pain or discomfort (sedation). A doctor or nurse will first check any risks for you about having the sedation. They will ask about your health, medical conditions, medicines and previous experiences with sedation or anaesthesia. This is to make sure that you are given sedation safely. They will also talk with you about the medicines they will use during your sedation, their risks and benefits, and what you can expect to be aware of during the colonoscopy and as you recover. Discuss any concerns or preferences with your clinician, including the option for no sedation.

Your sedation will be given according to current professional recommendations and guidelines and will take into account your risks. Your sedation may be given by a specialist anaesthetist, but this is not always required.

Cultural safety and equity

Involve Aboriginal and Torres Strait Islander Liaison Officers or Aboriginal and Torres Strait Islander Health Practitioners and Workers where available to address potential concerns about sedation.

Related resources

For clinicians and healthcare services

- [Guideline for the perioperative care of patients selected for day stay procedures \(PG15\)](#) - ANZCA
- [Guideline on procedural sedation \(PG09\)](#) - ANZCA
- [Position statement on informed consent for anaesthesia or sedation \(PS26\)](#) - ANZCA

Quality Statement 6 – Clinicians

A patient's colonoscopy is performed by a credentialed clinician working within their scope of clinical practice, who meets the requirements of an accepted certification and recertification process. Sedation or anaesthesia, and clinical support are provided by credentialed clinicians working within their scope of clinical practice.

For clinicians

Ensure that your training, skills and experience allow you to provide safe, high-quality care to a patient undergoing colonoscopy, in accordance with expected professional standards. Comply with your health service organisation's policies and procedures regarding your scope of clinical practice. Interact with your peers to ensure your performance, and theirs, meets the accepted requirements for safety and quality (for example, participate in peer-review meetings and quality clinical improvement processes – including the collection of quality indicators and reviews of evidence-based best practice).

If you are a colonoscopist, undergo certification and participate in a recertification process that is accepted by your professional association and employer. Supervise trainees at a level appropriate to their skill and experience.

If you are a clinician providing sedation, ensure you meet the Safe Sedation Competencies as described in ANZCA's *Guideline on procedural sedation (PG09)*.

For healthcare services

Identify credentials that are required for clinicians to perform colonoscopy or provide sedation or anaesthesia for patients undergoing colonoscopy, and ensure credentialing processes are adequate, as set out in *Credentialing health practitioners and defining the scope of clinical practice: A guide for managers and practitioners*.

For clinicians performing colonoscopy, identify accepted certification and recertification processes according to their clinical speciality and professional body and use this when credentialing clinicians and defining their scope of clinical practice. For trainee colonoscopists working towards certification, implement and ensure compliance with policies and procedures for the safe supervision of trainees. Ensure that non-anaesthetist clinicians who provide sedation fulfil the training and competencies outlined in the Safe Sedation Competencies and are targeting an appropriate level of sedation (minimal to moderate sedation), as described in ANZCA's *Guideline on procedural sedation (PG09)*. Support participation by clinicians in peer-review activities.

For consumers

When you have a colonoscopy, you can expect to be cared for by qualified clinicians who have met necessary health service organisation and professional requirements and standards. This includes the clinicians providing your nursing care, sedation or anaesthesia, and your colonoscopy. You can expect that the doctor or specialist nurse who carries out the colonoscopy will keep their skills and knowledge up to date.

Cultural safety and equity

Ensure clinicians proactively reflect on their assumptions and biases, and provide care that is holistic, culturally safe, and free of discrimination and racism (see recommendations at Building culturally safe systems).

Related resources

For clinicians and healthcare services

- [Guideline on procedural sedation \(PG09\)](#) – see Appendix IV for guidelines on competencies applicable to clinicians managing procedural sedation – ANZCA

Quality Statement 7 – Procedure

When a patient is undergoing colonoscopy their entire colon – including the caecum – is examined carefully and systematically. The adequacy of bowel preparation, clinical findings, biopsies, polyps removed, therapeutic interventions and details of any adverse events are documented. All polyps removed are submitted for histological examination.

For clinicians

To maximise adenoma detection, intubate the caecum or terminal ileum and allow adequate time for mucosal inspection whenever performing colonoscopy. In people with previous resection, examine the remaining bowel thoroughly. Assess the adequacy of bowel preparation using a validated tool such as the Boston Bowel Preparation Scale, Ottawa Bowel Preparation Scale, or the Aronchick Scale.

Document the quality of the bowel preparation, whether caecal intubation was achieved (with photodocumentation), withdrawal time, clinical findings, the details of polyps removed, how they were removed and whether they were retrieved. Ensure that all polyps removed are retrieved, where possible, and are sent for histopathology examination. For patients referred for colonoscopy due to a positive National Screening Program iFOBT result, reporting to the National Cancer Screening Register should occur as soon as possible.

Record adverse events, including perforation, post-polypectomy bleeding and sedation-related cardio-respiratory compromise in the patient record and relevant quality systems (for example, the facility's incident monitoring system). Inform the patient if adverse events have occurred and how they have been managed.

Equipment used to perform the colonoscopy should be of adequate quality to enable safe and accurate visualisation and photodocumentation of the bowel. Any adjunctive technologies (such as artificial intelligence [AI] polyp detection, foot pump irrigation and narrow band imaging) should be used in an assistive capacity and according to local policy. Clinicians remain accountable for delivering safe and high-quality care regardless of the technology used, and for ensuring their practice meets their professional obligations.

For healthcare services

Establish procedures to collect and periodically monitor the quality of colonoscopies at the service level, including caecal intubation, adenoma detection rate, sessile serrated lesion detection rate and adequacy of bowel preparation. Review and share organisation and clinician-level data findings with clinicians as part of quality monitoring and clinical quality improvement activities (such as clinical review meetings). Ensure that the number of patients booked on each list enables the colonoscopist to undertake a careful and systematic examination of each patient's colon. Review list sizes periodically as part of procedures to monitor the quality of colonoscopies at the healthcare service. Ensure a process is in place to act promptly and effectively on any results suggesting substandard quality.

Provide systems that require and support colonoscopists to maintain accurate records of the colonoscopy. This includes the adequacy of bowel preparation, biopsies taken, polyps removed and retrieved, all diagnostic and therapeutic interventions, details of any adverse events and procedure duration. Ensure complications and adverse events of colonoscopy are reported and monitored in the organisation's incident management system and other relevant systems and investigated appropriately.

Ensure that the standard of equipment provided for colonoscopy supports safe, high-quality colonoscopy, including for visualisation and photodocumentation. Ensure that organisational policies and procedures support the assessment of technology and equipment used for colonoscopy to identify the benefits and consequences of use, and appropriate clinical governance.

For consumers

Your colonoscopy will be performed to a high standard. During the colonoscopy, the whole length of your bowel will be carefully examined. This will make it more likely that bowel problems can be found and that growths such as polyps can be seen and removed. If bowel tissue or polyps are removed from your bowel they will be sent to pathology laboratories for examination under a microscope. All the records kept by health service organisations will have information about your colonoscopy, the

findings, and any complications that may have occurred during the procedure. You can ask to see this information if you want to.

Related resources

For clinicians and healthcare services

- [AI Implementation in Hospitals: Legislation, Policy, Guidelines and Principles, and Evidence about Quality and Safety](#) – the Commission
- [Position statement on artificial intelligence in healthcare](#) – Australian Medical Society [AMA]
- [Artificial intelligence \(AI\) and healthcare resources](#) – Australian Alliance for Artificial Intelligence in Healthcare
- [Meeting your professional obligations when using artificial intelligence in healthcare](#) – Australian Health Practitioner Regulation Agency [AHPRA]
- [Artificial intelligence – our work](#) – Royal Australian and New Zealand College of Radiologists [RANZCR]

Indicators

Indicator 7a	<u>Proportion of patients who had a colonoscopy whose bowel preparation was adequate using a validated assessment tool</u>
Indicator 7b	<u>Proportion of patients who had a colonoscopy whose entire colon was examined to the caecum and/or terminal ileum</u>
Indicator 7c	<u>Proportion of patients who had a colonoscopy that detected one or more adenoma(s)</u>
Indicator 7d	<u>Proportion of patients who had a colonoscopy that detected one or more sessile serrated lesion(s)</u>

Quality Statement 8 – Discharge

Following recovery and before discharge, the patient is advised verbally and in writing about the preliminary outcomes of the colonoscopy, the nature of any therapeutic interventions or adverse events, when to resume regular activities and medicines, and arrangements for medical follow-up. The patient is safely

discharged into the care of a responsible adult, in accordance with Australian and New Zealand College of Anaesthetists guidelines.

For clinicians

Before discharge, the responsible clinician or their delegate should talk to the patient and briefly describe what happened during the colonoscopy. This includes whether the colonoscopy was completed satisfactorily, initial observations, whether biopsies or polypectomies were performed, and if any adverse events occurred. Advise patients of any arrangements for follow-up medical consultation and when final results and recommendations will be provided to them and their referring clinician.

Ensure patients are discharged by authorised clinical personnel into the care of a responsible adult after satisfactory discharge criteria are met. If, despite all reasonable efforts, a responsible adult is not available, the clinician responsible for managing sedation may exercise their judgement in deciding alternative post-sedation supervision and transport (excluding driving or public transport). Any exception should take into account the nature and duration of the sedation, and the patient's recovery and risk of adverse events, in accordance with ANZCA's *Guideline on procedural sedation (PG09)*. Alternatives, including admission or scheduling of appointments to allow longer periods in recovery, should be explored when scheduling admissions to avoid denying colonoscopy to people due to their social circumstances.

Provide instructions about early post-procedure care and resumption of normal activities, including making legally binding decisions, operating machinery and resuming regular medication. Advise patients of what to do if they experience symptoms suggesting a complication of the colonoscopy and provide them with specific contact details for obtaining appropriate advice. Any information given verbally about the procedure or post-discharge should also be provided in written format. Consider admission for a patient at high risk of an adverse outcome who is otherwise not suitable for discharge.

For healthcare services

Ensure that policies and procedures for monitoring, supervising and discharging patients align with current recommendations for post-operative care following sedation or anaesthesia (for example, ANZCA's *Guideline for the perioperative care of patients selected for day stay procedures [PG15]*).

Ensure that procedures are in place for discharging patients into the care of a responsible adult. Exceptions to post-sedation supervision by a responsible adult and

transport to their discharge destination (excluding driving or public transport) should be based on the clinical judgement of the clinician managing sedation. Exceptions should take into account the depth of sedation and be in accordance with ANZCA guidelines.

Provide written instructions about early post-procedure care and resumption of normal activities, including medicines. Ensure that there is a response plan for patients in the event of problems arising post-discharge. Provide patients with discharge information, including specific health service contact details after hours.

Pre-admission procedures should identify patients who genuinely cannot identify a responsible adult to accompany them home and stay with them overnight and allow for suitable arrangements to be made, according to their risk. Policies should allow for extended recovery periods and overnight admission, if needed, for patients who have comorbidities and cannot be cared for adequately at home in the immediate period post-discharge or who do not meet discharge criteria (as appropriate to the type of facility).

For consumers

After your colonoscopy, you will be cared for while you recover from the sedation. Before you go home, a doctor or nurse will tell you what happened during the colonoscopy, whether any polyps or other tissue were removed and whether there were any problems during the procedure. They will tell you about any arrangements or follow-up appointments you need to make. You may find it difficult to remember this information so it will be also given to you in writing. You will also be provided a copy of your colonoscopy report (see Quality statement 9 – Reporting and follow-up).

You will be able to go home once your doctor or nurse is satisfied that you have recovered from the sedation. You should not drive and should have an adult to accompany you home. It is also recommended that you have someone stay with you on the night after the colonoscopy. If this is not possible, discuss this with your clinician before you have the colonoscopy.

You will be given written instructions on how to care for yourself when you go home and when to start your regular medicines and diet again. You will be provided with information about what to do if you have any problems after going home, including a phone number that you can call after hours.

Cultural safety and equity

Ensure that information given to patients is provided in a way that the patient understands and is culturally safe. Allow time for explanation and questions. Use plain language and visual aids where appropriate. Involve family or kin, Aboriginal and Torres Strait Islander Liaison Officers and translators where needed.

Related resources

For clinicians and healthcare services

- [Guideline for the perioperative care of patients selected for day stay procedures \(PG15\)](#) – ANZCA
- [Guideline on procedural sedation \(PG09\)](#) – ANZCA

Quality Statement 9 – Reporting and follow-up

The colonoscopist communicates the reason for the colonoscopy, its findings, any histology results and recommendations for follow-up in writing to the general practitioner, any other relevant clinician and the patient. This information is recorded in the facility records and other electronic shared record management systems to enable accurate follow-up by other clinicians. Recommendations for surveillance colonoscopy, if required, align with national evidence-based guidelines. If more immediate treatment or follow-up is needed, the colonoscopist makes appropriate arrangements.

For clinicians

Ensure a colonoscopy report is provided to the patient, their GP and any other relevant clinicians that includes:

- the reason for the colonoscopy
- findings during the colonoscopy examination
- histology results where relevant
- recommendations for follow-up based on national evidence-based guidelines.

Ensure that both positive and negative histology findings are communicated. The need and time interval for future screening and surveillance colonoscopies should be guided by evidence-based guidelines, such as the Cancer Council Australia's *Clinical practice guidelines for the prevention, early detection and management of colorectal cancer*, and *Clinical practice guidelines for surveillance colonoscopy*. If prompt treatment or investigation is required (such as for histologically confirmed colorectal cancer or high-risk lesions), make the necessary arrangements and ensure these are communicated to the patient and their referring clinician. If presenting symptoms

have not been explained by the colonoscopy, advise the patient and refer on if further investigation or treatment is required.

Upload the report and results to the patient's healthcare record and My Health Record (if this capability is available in clinical information systems). For National Screening Program participants, report colonoscopy outcomes, results and adverse events to the National Cancer Screening Register.

For healthcare services

Ensure that policies and procedures clearly delineate responsibilities for managing patient recall and follow-up for the colonoscopist, the health service and the GP. Ensure that policies and procedures for information management and communication reflect these arrangements. To ensure complete reporting of colonoscopy, policies should:

- include arrangements for the reporting of all histology results if any tissue was removed, regardless of the histological findings
- ensure surveillance intervals are updated based on histology results
- ensure histology results, updated surveillance intervals and other recommendations are uploaded to the facility records and other shared record systems such as the My Health Record
- ensure the colonoscopy report, histology outcomes and surveillance intervals are provided to referring clinicians (GPs), other relevant clinicians and the patient
- support reporting to the National Cancer Screening Register for patients referred through participation in the National Screening Program.

Ensure systems are in place for the prompt communication and management of histologically confirmed colorectal cancer or high-risk lesions. Support and promote clinicians' use of national evidence-based guidelines, such as the Cancer Council Australia's *Clinical practice guidelines for the prevention, early detection and management of colorectal cancer* and *Clinical practice guidelines for surveillance colonoscopy*, when making recommendations for future surveillance and follow-up.

For consumers

The results of your colonoscopy will be given to you, your general practitioner, and any of your other doctors who may need to be informed. The results can also be added to your electronic health record, known as the My Health Record. The letter or report will say why you had the colonoscopy, what was found, whether any tissue or growths (such as polyps) were removed from your bowel and sent for testing, and the results of those tests.

The report will also say whether you need to go and see a doctor for a follow-up visit, have further tests or treatment or another colonoscopy in the future, and when these

should happen. These recommendations will be different for each person and will depend on your medical and family history and what was found during the colonoscopy.

Cultural safety and equity

Ensure documentation, including results, follow-up and future management, is provided to the referring primary healthcare provider or ACCHO in a timely fashion.

Ensure clinicians have sufficient cultural competence to support Aboriginal and Torres Strait Islander peoples’ participation in bowel cancer prevention and treatment and use culturally appropriate materials.

Related resources

For clinicians and healthcare services

- [Forms for healthcare providers](#) - National Bowel Cancer Screening Program
- [Healthcare provider portal](#) for health professionals to access and submit screening data for the bowel and cervical screening programs - National Cancer Screening Register
- [Value In Care – Optimising Surveillance Colonoscopy \[VIC-COL\] multi-component quality improvement strategy tools](#) - Monash University
- [Clinical practice guidelines for the prevention, early detection and management of colorectal cancer](#) - Cancer Council Australia
- [Clinical practice guidelines for surveillance colonoscopy](#) - Cancer Council Australia

For consumers

- [Check your cancer risk](#) online tool - Cancer Australia
- [Lifestyle and Risk Reduction](#) - webpage for practical tips and information - Cancer Australia

Indicators

The Commission has developed a set of indicators to support clinicians and healthcare services to monitor how well they are implementing the care recommended in this Clinical Care Standard. The indicators are to ensure information about a person's medicines is recorded and shared to enable accurate follow-up	Indicator 9a Evidence of local medicines use to ensure information about a person's medicines is recorded and shared to enable accurate follow-up
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When using the indicators, please refer to the definitions required to collect and calculate indicator data which are specified online at [Meteor](#).

You can find a description of each indicator below with links to its individual specifications.

List of indicators

Indicator 2a	Evidence of a locally approved policy that ensures the timely and appropriate provision of colonoscopy
Indicator 7a	Proportion of patients who had a colonoscopy whose bowel preparation was adequate using a validated assessment tool
Indicator 7b	Proportion of patients who had a colonoscopy that detected one or more adenoma(s)
Indicator 7d	Proportion of patients who had a colonoscopy that detected one or more sessile serrated lesion(s)
Indicator 9a	Evidence of local arrangements to ensure information about a person’s colonoscopy is recorded and shared to enable accurate follow-up

Cultural safety and equity for Aboriginal and Torres Strait Islander peoples

Health outcomes for Aboriginal and Torres Strait Islander peoples can be improved by addressing systemic racism and other root causes that reduce access to care. Historical and current contributing factors include a lack of culturally safe care, culturally appropriate health education and sociocultural determinants such as differences in employment opportunities.

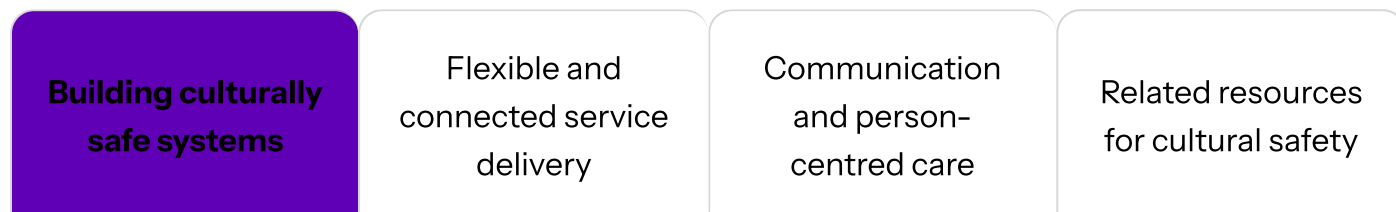
The considerations for improving cultural safety and equity in this Clinical Care Standard focus primarily on overcoming cultural power imbalances and improving outcomes for Aboriginal and Torres Strait Islander people through better access to health care

Cultural safety and equity recommendations in this document have been developed in consultation with Aboriginal and Torres Strait Islander individuals, clinicians and representative health service organisations. However, it is recognised that cultural safety is determined by the Aboriginal and Torres Strait Islander individuals, families and communities experiencing the care.

Recommendations

When implementing this Clinical Care Standard, cultural safety can be improved through embedding an organisational approach such as described in the recommendations below.

When providing care for Aboriginal and Torres Strait Islander people, particular consideration should be given to the following recommendations.



- Ensure systems and processes support people to self-report their Aboriginal and Torres Strait Islander status and to record self-identification.
- Ensure all staff engage regularly in cultural safety training.
- Disaggregate data by Aboriginal and Torres Strait Islander status when using the indicators included in this Standard. This will support identification of access and outcome issues so that improvements can be made.
- Implement the six actions for Aboriginal and Torres Strait Islander Health from the NSQHS Standards.
- Provide flexible service delivery to optimise attendance and help develop trust with Aboriginal and Torres Strait Islander individuals and communities.
- Establish robust communication channels and referral pathways with primary healthcare providers, including Aboriginal and Torres Strait Islander Community Controlled Health Organisations (ACCHOs).
- Where possible, provide outreach services close to home, on Country or in collaboration with ACCHOs or other community healthcare providers.
- Take a collaborative approach to ensure that interventions are suitably tailored to the individual's personal needs and preferences for care.
- Encourage the inclusion of support people, family and kin, and the person's trusted healthcare provider (such as their ACCHO) in all aspects of care, including decision making and plans for treatment and management.
- Engage culturally appropriate interpreter services and cultural translators when this will assist the patient.
- Involve Aboriginal and Torres Strait Islander Health Workers or Aboriginal and Torres Strait Islander Health Practitioners as part of a patient's multidisciplinary team and involve Aboriginal and Torres Strait Islander Liaison Officers in hospital settings.

- Use culturally and linguistically appropriate materials to aid in communication and discussion, accounting for varying levels of health literacy.
- *[NSQHS Standards User Guide for Aboriginal and Torres Strait Islander Health](#)* – a guide to help improve the quality of care and health outcomes for Aboriginal and Torres Strait Islander people based on the National Safety and Quality Health Service Standards.
- *[National Agreement on Closing the Gap](#)* – an agreement built around four priority reforms for transforming the way governments work with, and for, Aboriginal and Torres Strait Islander peoples to improve outcomes.
- *[Cultural respect framework 2016–2026 for Aboriginal and Torres Strait Islander health](#)* – a framework that commits the Australian Government and all states and territories to embed cultural respect principles into their health systems.
- *[Clinical Yarning](#)* – a patient-centred framework to improve communication in Aboriginal health care.
- *[Communicating positively: a guide to appropriate Aboriginal terminology](#)* – a guide developed by NSW Health to use when working with Aboriginal people and communities, and when developing policy and programs.
- *[Plain English Health Dictionary](#)* – a resource developed by the Northern Territory Government as a resource for Aboriginal Interpreter Services.

Additional recommendations are provided for some [Quality statements](#).

Resource hub

Implementation resources are resources developed by the Commission that will assist in implementing and understanding the Clinical Care Standards. They include short guides to the Standards for consumers, clinicians and healthcare services, and other tools and resources to support implementation.

Related resources are other resources that the Commission has identified as relevant and useful. Most often, these come from sources outside the Commission. They may include additional information, guidelines, tools and consumer materials.

For clinicians and healthcare services

Implementation resources

[Information for clinicians - Colonoscopy Clinical Care Standard →](#)

[Information for healthcare services - Colonoscopy Clinical Care Standard →](#)

Self-assessment tool

This Self-Assessment Tool can be used by healthcare services to ensure they are providing quality colonoscopy services in line with the requirements of the Standard.

[Self-Assessment Tool – Colonoscopy Clinical Care Standard →](#)

Informed consent for colonoscopy

Quality statement 3 in the Standard states that a patient should receive adequate information and provide informed consent before starting bowel preparation. Find out what informed consent means for colonoscopy:

[Informed Consent - Fact sheet for clinicians →](#)

[NSQHS Standards Implementing the Colonoscopy Clinical Care Standard – Informed consent →](#)

Templates for referral, reporting and follow-up letters

Templates and guidance to assist in drafting referral, reporting and follow-up letters.

[Colonoscopy Referral Information - fact sheet and referral template →](#)

[Colonoscopy Report Template →](#)

Related resources

Clinical practice guidelines and pathways

- [Clinical practice guidelines for the prevention, early detection and management of colorectal cancer](#) - Cancer Council Australia
- [Clinical practice guidelines for surveillance colonoscopy](#) - Cancer Council Australia
- [Optimal care pathways: Colorectal cancer](#) - a framework for delivery of high-quality evidence-based care for people with cancer - Cancer Council Victoria and Department of Health Victoria
- [NSW colonoscopy categorisation](#) - NSW Agency for Clinical Innovation
- [Colonoscopy categorisation guidelines](#) - Victoria Department of Health

State and territory-based guidance

- [NSW colonoscopy categorisation](#) - NSW Agency for Clinical Innovation
- [Colonoscopy categorisation guidelines](#) - Victoria Department of Health

Bowel preparation

- [Sodium glucose co-transporter 2 inhibitors | Safety advisory – diabetic ketoacidosis and surgical procedures](#) - Therapeutic Goods Administration [TGA]
- [Periprocedural diabetic ketoacidosis \(DKA\) with SGLT2 inhibitor use in people with diabetes](#) - Australian Diabetes Society [ADS]
- [Clinical Practice Recommendations regarding patients taking GLP-1 receptor agonists and dual GLP-1/GIP receptor co-agonists prior to anaesthesia or sedation for surgical and endoscopic procedures](#) - ANZCA, GESA, National Association of Clinical Obesity Services [NACOS], ADS

Sedation and anaesthesia

- [Guideline for the perioperative care of patients selected for day stay procedures \(PG15\)](#) - ANZCA
- [Guideline on procedural sedation \(PG09\)](#) - ANZCA
- [Position statement on informed consent for anaesthesia or sedation \(PS26\)](#) - ANZCA

Artificial Intelligence guidance

- [Position statement on artificial intelligence in healthcare](#) - Australian Medical Society [AMA]
- [Artificial intelligence \(AI\) and healthcare resources](#) - Australian Alliance for Artificial Intelligence in Healthcare
- [Meeting your professional obligations when using artificial intelligence in healthcare](#) - Australian Health Practitioner Regulation Agency [AHPRA]
- [Artificial intelligence – our work](#) - Royal Australian and New Zealand College of Radiologists [RANZCR]

Auditing and National Bowel Cancer Screening Program Reporting

- [Forms for healthcare providers](#) - National Bowel Cancer Screening Program
- [Healthcare provider portal](#) for health professionals to access and submit screening data for the bowel and cervical screening programs - National Cancer Screening Register
- [Value In Care – Optimising Surveillance Colonoscopy \[VIC-COL\] multi-component quality improvement strategy tools](#) - Monash University

Assessment to the NSQHS Standards

Implementation resources

Implementing the Colonoscopy Clinical Care Standard is a requirement for all health service organisations assessed against the National Safety and Quality Health Service Standards for their accreditation.

Advisory

Advisory [AS18/12: Implementing the Colonoscopy Clinical Care Standard](#) describes the assessment requirements for Actions [1.23](#), [1.24](#), [1.27b](#), and [1.28a](#) of the National Safety and Quality Health Service (NSQHS) Standards (second edition) for health service organisations implementing the Colonoscopy Clinical Care Standard.

Assessor video - what you need to know about the Colonoscopy Clinical Care Standard

Video ignored

Credentialing, certification and re-certification of colonoscopists

Certification (of training) and recertification (of ongoing competency) in adult colonoscopy are mandatory for all practitioners working in health service organisations being assessed to the National Safety and Quality Health Service Standards (second edition). Download the following fact sheet for information for clinicians on certification and re-certification.

[Certification and Recertification of practising adult colonoscopists - NSQHS Standards →](#)

For consumers

Implementation resources

[Guide for consumers - Colonoscopy Clinical Care Standard →](#)

What you need to know before you have a colonoscopy

This video explains more about your rights, choices and what you need to do if you are having a colonoscopy and why preparing your bowel is so important.

Video ignored

Related resources

Colonoscopy and Bowel preparation

- [Information about colonoscopy](#) – a fact sheet for patients – Gastroenterological Society of Australia [GESA]
- [Information about bowel preparation](#) – a fact sheet that provides information for patients having a colonoscopy – GESA

Bowel cancer risk

- [Check your cancer risk](#) online tool – Cancer Australia
- [Lifestyle and Risk Reduction](#) – webpage for practical tips and information – Cancer Australia

For Aboriginal and Torres Strait Islander peoples

Implementation resources

The Commission has developed culturally appropriate resources with Aboriginal and Torres Strait Islander peoples to provide useful information about having a colonoscopy procedure.

These resources aim to help Aboriginal and Torres Strait Islander patients understand their rights, choices and what they need to do if they are referred for a colonoscopy.

Healthcare professionals and health service organisations working with Aboriginal and Torres Strait Islander communities are encouraged to discuss and share these resources with patients, their carers and with colleagues.

They are based on guidance from the Commission's national *Colonoscopy Clinical Care Standard*, which describes the safe, appropriate and high-quality use of colonoscopy.

Consumer fact sheet

The fact sheet provides useful information on each step of a colonoscopy procedure. It discusses each stage from referral to discussion of the colonoscopy results.

Colonoscopy for Aboriginal and Torres Strait Islander peoples – Fact sheet →

Consumer video

This video explains an Aboriginal and Torres Strait Islander person's rights, choices and what they need to do if they are referred for a colonoscopy. It outlines why emptying the bowel properly before a colonoscopy is so important.

Video ignored

Related resources

- [Culturally appropriate bowel screening resources](#) - SA Health
- [NSQHS Standards User guide for Aboriginal and Torres Strait Islander health](#) - A guide to help improve the quality of care and health outcomes for Aboriginal and Torres Strait Islander people based on the National Safety and Quality Health Service Standards.
- [National Agreement on Closing the Gap](#) - An agreement built around [four priority reforms](#) for transforming the way governments work with, and for, Aboriginal and Torres Strait Islander peoples to improve outcomes.
- [Cultural respect framework 2016–2026 for Aboriginal and Torres Strait Islander health](#) - A framework that commits the Australian Government and all states and territories to embed cultural respect principles into their health systems.
- [Clinical Yarning](#) - A patient-centred framework to improve communication in Aboriginal health care.
- [Communicating positively: a guide to appropriate Aboriginal terminology](#) - A guide developed by NSW Health to use when working with Aboriginal people and communities, and when developing policy and programs.
- [Plain English Health Dictionary](#) - A resource developed by the Northern Territory Government as a resource for Aboriginal Interpreter Services.

Bowel cancer prevention

Modifiable lifestyle risk factors contribute to more than 50% of bowel cancer cases.

Before or after a colonoscopy procedure or a bowel cancer screening test, is an ideal time to talk with patients about lifestyle changes. Our fact sheet and poster can help start the conversation.

These resources have been endorsed by the Gastroenterological Society of Australia (GESA) the Gastroenterological Nurses College of Australia (GENCA), the Royal Australasian College of Surgeons (RACS) and the Colorectal Surgical Society of Australia and New Zealand (CSSANZ).

Related resources are also provided within the relevant [quality statement](#).

Communication resources

Show your support for the updated *Colonoscopy Clinical Care Standard* by sharing our resources on your website, social networks or within your health service organisation.

Our communications kit includes newsletter copy and social media graphics to help you share and promote the standard within your networks.

[View communications kit](#)

Launch of the Standard

The revised standard was launched on 21 September 2025 alongside the Atlas Focus Report: Colonoscopy at the World Congress of Gastroenterology at Australian Gastroenterology Week conference held in Melbourne.

[Find out more on the Colonoscopy Hub](#)

Videos

CEO Presentation

Video

[Watch now →](#)

Expert Panel Discussion

Video

[Watch now →](#)

This panel of experts on colonoscopy discussed the impact of the Clinical Care

Commission Chief Executive Officer, Conjoint Professor Anne Duggan launched the updated Clinical Care Standard and Atlas healthcare variation findings at the World Congress of Gastroenterology at Australian Gastroenterology Week 2025 conference in Melbourne.

Standard, the ongoing disparity in access to colonoscopy across Australia, and the challenges in communication across the health system at the World Congress of Gastroenterology at Australian Gastroenterology Week 2025 conference in Melbourne.

Story of Excellence

Strengthening foundations for auditing and oversight in colonoscopy care

Northern NSW Local Health District Clinical Operations Support Manager, Sally Smith, outlines why the LHD received a Story of Excellence Award for effectively implementing the *Colonoscopy Clinical Care Standard*.

Video ignored

[Read their story](#)

Updates to the Standard

The Standard was first published in 2018 and was updated in 2025 to ensure continued alignment with the current evidence base and expected practice. While there has been no major change to the recommended guidance for colonoscopy, key updates to the standard include:

- clarified requirements to improve communication at referral (Quality statement 1)
- the addition of considerations for open access colonoscopy services, including for referral, provision of information and informed consent processes (Quality statement 1)
- updated information regarding sedation options and discharge supervision (Quality statement 5)
- the addition of photodocumentation as a minimum requirement for the procedure (Quality statement 7)
- strengthened requirements for ensuring shared, accurate records following colonoscopy, such as use of My Health Record, so that patients are offered

appropriate follow-up treatment or surveillance in accordance with evidence-based guidelines. For patients referred through the National Bowel Cancer Screening Program, report into the National Cancer Screening Register (Quality statement 9)

- addition of cultural safety and equity considerations
- amendments to the indicators and the addition of 2 new indicators.

Further information on changes can be found on page 4 of the Standard.

More about the Standard

What is the background to the Standard?

The Australian Commission on Safety and Quality in Health Care developed a safety and quality model for colonoscopy in 2016–17, supported by funding from the Australian Government Department of Health and Aged Care.

The model comprises three elements:

- a *Colonoscopy Clinical Care Standard* (first published in 2018)
- implementation of the Standard through the National Safety and Quality Health Service (NSQHS) Standards in public and private hospitals, and day procedure services
- certification – and periodic recertification – of colonoscopists' performance in accordance with defined quality indicators and performance targets established by the Gastroenterological Society of Australia (GESA), and overseen by the Conjoint Committee for the Recognition of Training in Gastrointestinal Endoscopy (CCRTGE) and the Recertification in Colonoscopy Conjoint Committee (RCCC).

Development and consultation on the model

The draft safety and quality model for colonoscopy was developed through a consultation process and a review of Australian and international experiences, from October 2015 to July 2016. The consultation included consumers, clinicians and health service operators, and representatives of the Gastroenterological Society of Australia (GESA), the Colorectal Surgical Society of Australia and New Zealand (CSSANZ), and the Royal Australasian College of Surgeons (RACS).

Stakeholders identified the need for the colonoscopy safety and quality model to address:

- the appropriateness of the procedure and adenoma detection
- facilities where colonoscopy is performed
- the training and performance of colonoscopists
- the training and performance of other members of the clinical team (including nurses and anaesthetists)
- processes that support monitoring of colonoscopist procedure volume, caecal intubation, adenoma detection rates and serious adverse events.

Following agreement on the model, The *Colonoscopy Clinical Care Standard* was developed by the Commission and first released in 2018. It was determined that the *Colonoscopy Clinical Care Standard* would be implemented in public and private hospitals and day procedure centres, and its implementation required under the National Safety and Quality Health Service (NSQHS) Standard 1 – Governance for Safety and Quality in Health Service Organisations.

Read more about the scope and goal of this Standard or see further background in the [*Colonoscopy Clinical Care Standard*](#).

Where does the Standard apply?

Implementing the *Colonoscopy Clinical Care Standard* is a requirement for all health service organisations assessed against the National Safety and Quality Health Service Standards.

More information and resources can be found in [Assessment to the NSQHS Standards](#).

This Standard applies to care provided in:

- primary care, including general practice (specifically in regard to referral)
- other specialists' rooms
- private hospitals
- public hospitals
- day procedure services.

This Standard is particularly relevant to:

- Aboriginal and Torres Strait Islander Health Workers and Aboriginal and Torres Strait Islander Health Practitioners
- anaesthetists
- colorectal and general surgeons performing colonoscopy
- clinicians undertaking colonoscopy as part of a training program
- gastroenterologists

- general practitioners (GPs), including rural generalists
- health service managers
- non-anaesthetist sedationists
- nurses and nurse practitioners.

Not all quality statements within this Clinical Care Standard will be applicable to every healthcare service or clinical unit. Healthcare services should consider their individual circumstances in determining how to apply the statement.

When implementing this Standard, healthcare services should consider:

- the context in which care is provided
- local variation
- quality improvement priorities of the individual healthcare service.

Healthcare services in rural and remote settings may need different strategies to implement the standard. For example, the use of:

- hub-and-spoke models integrating larger and smaller health services and Aboriginal and Torres Strait Islander Community Controlled Health Organisations (ACCHOs)
- telehealth consultations
- multidisciplinary teams, including GPs and nurses where clinically appropriate.

The *Colonoscopy Clinical Care Standard* relates to the care of adult patients undergoing colonoscopy for screening, diagnosis, treatment or surveillance. It covers the period from when a patient is referred for consideration of colonoscopy through to discharge, including planning for follow-up care.

How does the Standard support cultural safety and equity?

The Commission is committed to supporting healthcare services to provide culturally safe and equitable healthcare to all Australians. See our recommendations on culturally safe care from the *Colonoscopy Clinical Care Standard*.

Person-centred care recognises and respects differences in individual needs, beliefs, and culture. The Commission:

- is committed to supporting healthcare services to provide culturally safe and equitable healthcare to all Australians
- acknowledges that discrimination and inequity are significant barriers to achieving high-quality health outcomes for some patients from culturally and linguistically diverse communities

Culturally safe service provision and environments are those where the places, people, policies and practices foster mutual respect, shared decision making, and an understanding of cultural, linguistic and spiritual perspectives and differences. Cultural safety is supported by organisations and individuals that recognise cultural power imbalances and actively address them by:

- ensuring access to and use of interpreter services or cultural translators when this will assist the patient and aligns with their wishes
- providing visual or written information in a language that the patient, their family and carers will understand
- providing cultural competency training for all staff
- encouraging clinicians to review their own beliefs and attitudes when treating and communicating with patients
- identifying variation in healthcare provision or outcomes for specific patient populations, including those based on ethnicity, and responding accordingly.

See the specific recommendations on cultural safety for Aboriginal and Torres Strait Islander peoples for this Standard.

Which key organisations have endorsed the Standard?

The *Colonoscopy Clinical Care Standard* has been endorsed by 18 key organisations:

- Australian and New Zealand College of Anaesthetists
- Australian College of Rural and Remote Medicine
- Cancer Council Australia
- Colorectal Surgical Society of Australia and New Zealand
- Day Hospitals Australia
- Gastroenterological Nurses College of Australia
- Gastroenterological Society of Australia
- Royal Australasian College of Surgeons
- Australian College of Nurses
- Australian College of Nurse Practitioners
- Australian College of Perioperative Nurses
- The Australasian College of PeriAnaesthesia Nurses
- Crohn's & Colitis Australia
- Inherited Cancers Australia
- National Aboriginal Community Controlled Health Organisation
- Royal Australasian College of Medical Administrators
- Royal College of Pathologists of Australasia
- Rural Doctors Association of Australia.

Who was consulted on the Standard's development?

The Commission develops Clinical Care Standards taking into account:

- advice from multidisciplinary topic working groups which include clinicians, consumers, and researchers
- consultation with key stakeholders including consumer bodies, professional organisations, and state and territory health departments.

The **Colonoscopy Clinical Care Standard Topic Working Group** provided expert advice on the development and review of the Standard. In addition, a targeted consultation process was conducted with key stakeholders.

Colonoscopy Clinical Care Standard - Topic Working Group Membership List

Many members of the Colonoscopy Clinical Care Standard Topic Working Group were involved in the original development of the standard in 2018.

- advise on the continued scope and key components of care within the Standard
- advise on the key sources of evidence to inform the review
- advise on revisions to quality statements and supporting indicators
- recommend strategies to support the implementation of the updated Standard
- actively support raising awareness of the updated Standard.

The main roles of the Topic Working Group were to:

All topic working group members are required to disclose financial, personal and professional interests that could, or could be perceived to, influence a decision made, or advice given to the Commission. Disclosures are managed in line with the [Commission's Policy on Disclosure of Interests](#).

What was the evidence base for this Standard?

Further information is available on the evidence sources underpinning the *Colonoscopy Clinical Care Standard (2025)*.

Evidence Sources: Colonoscopy Clinical Care Standard 2025 →